

PACKAGE INSERT

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HOVID ALLOPURINOL TABLET

VIALLO3-var (MY)

DESCRIPTION

Round, white uncoated tablet, bevel-edged, flat faces, "HD" and break-bar embossed on the same face.

COMPOSITION

Each tablet contains:
Allopurinol 100 mg / tablet.

PHARMACODYNAMICS

Allopurinol is a structural isomer of hypoxanthine. The affinity of allopurinol for xanthine oxidase is much greater than that of xanthine. Allopurinol is of value in the treatment of gout because it is an inhibitor of xanthine oxidase, the enzyme responsible for converting xanthine and hypoxanthine to uric acid.

Both Allopurinol and its major metabolite alloxanthine (oxipurinol) inhibit xanthine oxidase. Xanthine and hypoxanthine are more easily excreted than uric acid. Allopurinol decreases the rate of formation and the blood levels of uric acid considerably but does not appreciably affect that of the more water soluble xanthine. Body uric acid is therefore decreased by reducing uric acid formation. By lowering the serum concentration of uric acid below its solubility limits, allopurinol promotes the resolution of tophi and uric acid crystals.

PHARMACOKINETICS

Absorption: Allopurinol is rapidly absorbed from the upper gastrointestinal tract and appears to be about 80% to 90% absorbed.

Blood concentration: 100 mg of Allopurinol administered thrice daily produced plasma concentrations of alloxanthine (oxipurinol), the major metabolite of allopurinol, of 5 to 15 microgram/ml, after one week.

Half-life: Plasma half-life is about 1 to 3 hours for allopurinol and 12 to 30 hours for alloxanthine (oxipurinol).

Protein Binding: Neither allopurinol nor its metabolite, oxipurinol, is bound to plasma proteins.

Distribution: Allopurinol is well distributed throughout the body water. Both allopurinol and alloxanthine (oxipurinol) are not bound to serum proteins and are excreted mainly in urine. Alloxanthine (oxipurinol) may accumulate in renal function impairment.

Metabolism: Allopurinol is rapidly oxidized in the liver to its active metabolite, alloxanthine (oxipurinol).

Excretion: About 30% of a dose is excreted unchanged in the urine, whilst 45 to 65% is excreted as alloxanthine (oxipurinol).

INDICATIONS

For treatment or prevention of:

- Chronic primary or secondary gout.
- Uric acid nephropathy.
- Uric acid stone formation.
- Problems associated with hyperuricemia (i.e. tissue urate deposition, renal calculi, or acute urate nephropathy) secondary to cancer chemotherapy or radiation therapy.

CONTRAINDICATIONS

- It should not be used during acute attack of gout.
- Use is not recommended in nursing mothers.

WARNINGS AND PRECAUTIONS

- Caution in patients with idiopathic hemochromatosis, and renal or hepatic function impairment.
- Treatment should be stopped if any skin reactions develop.
- A high fluid intake (no less than 2.5 to 3 litres daily) and maintenance of a slightly alkaline urine are recommended.
- Risk of precipitating acute gout during initial therapy is reduced by prophylactic doses of colchicine.

- Complete blood counts should be performed particularly during first month of therapy.
- Allopurinol should be discontinued at the first appearance of skin rash or other signs which may indicate an allergic reaction. Hypersensitivity to allopurinol usually appears after some weeks of therapy, and more rarely immediately after beginning treatment.
In some instances, a skin rash may be followed by severe reactions such as exfoliative, urticarial and purpuric lesions as well as Stevens-Johnson syndrome, and / or generalised vasculitis, irreversible hepatotoxicity and even death.

PREGNANCY AND LACTATION

Safety for use in pregnancy has not been established.

DRUG INTERACTIONS

- Dosage adjustments may be necessary when used concurrently with alcohol, chlorthalidone, ethacrynic acid, frusemide, metolazone, pyrazinamide, quinethazone, thiazide diuretics, mercaptopurine, azathioprine, anticoagulants, coumarin or indandione derivative.
- Co-administration of ampicillin or cyclophosphamide may increase the potential for toxicity.
- Effects of urinary acidifiers may increase the possibility of kidney stone formation during concurrent therapy with allopurinol.

MAIN SIDE/ADVERSE EFFECTS

Hypersensitivity reactions (skin rash), nausea, vomiting, abdominal pain, diarrhoea and drowsiness.

OVERDOSE AND TREATMENT

Clinical features: Nausea, vomiting, abdominal pain, diarrhoea, headache, drowsiness.

Treat overdosage by emesis or gastric lavage, if appropriate; with the necessary symptomatic and supportive measures, if required.

DOSAGE AND ADMINISTRATION

Adults

- **Antigout:** Oral, 100 to 200 mg two or three times a day; or 300 mg as a single dose once a day.
- **Antiurolithic:** Oral, 100 to 200 mg one to four times a day; or 300 mg as a single dose once a day.
- **Usual prescribing limits:** Up to 800 mg daily.

Children

For use in secondary hyperuricaemia associated with malignancies.

- **Up to 6 years:** Oral, 50 mg three times a day.
- **6 to 10 years:** Oral, 100 mg three times a day, or 300 mg as a single dose once a day.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage:
Store below 30 °C.

Presentation/Packing:
Tablet 100 mg x 100's, 1000's, Blisters 10 x 10's.
(Not all presentations available locally)

Product Registration Holder: HOVID Bhd.
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Manufactured by: HOVID Bhd.
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